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LAB REPORT

NAME	: Mr.SUDHAKAR SAINATHAN	**UHID NO/Visit ID**	: TND011.00097304/TND011.113617
Age/Gender	: 37 Y 1 M 25 D/M	**Collected**	: 10/Feb/2026 09:03AM
Ref Doctor	: SELF	**Received**	: 10/Feb/2026 12:53PM
Ref. Cust	: PHLB_1697073	**Reported**	: 10/Feb/2026 02:42PM
Client Code	: TND011	**Barcode**	: MB1893562

MEDIBUDDY FULL BODY HEALTH CHECKUP

LIPID PROFILE , SERUM

Test Name	Result	Unit	Bio. Ref. Range	Method
Total Cholesterol	171	mg/dL	Desirable : < 200 Borderline High :200 - 239 High : ≥ 240	Cholesterol Oxidase Peroxidase (CHOD-PAP)
HDL Cholesterol	**35.8**	mg/dL	> 40	Homogenous Enzymatic Colorimetric
Total Triglycerides	126.8	mg/dL	Desirable Level : < 150 Borderline : 150 - 199 High : 200 - 499 Very High : 500	GPO-POD
VLDL Cholesterol	25.36	mg/dL	≤ 30	Calculated
LDL Cholesterol	**109.84**	mg/dL	Optimal: < 100 Above Optimal: 100 - 129 Borderline: 130 - 159 High: 160 - 189 Very High: >= 190	Calculation
Non - HDL Cholesterol	**135.2**	mg/dL	≤ 130	Calculated
Chol / HDL Ratio	**4.8**		Low Risk: 3.3 - 4.4 Average Risk: 4.5 - 7.1 Moderate Risk: 7.2 - 11.0 High Risk : > 11.0	Calculated
TGL / HDL Ratio	**3.5**		Optimal: < 2.5 Mild to moderate risk: 2.5 - 5.0 High Risk: > 5.0	Calculated
HDL / LDL Ratio	0.3			Calculated
LDL / HDL Ratio	**3.1**		Desirable: 0.5 - 3.0 Borderline Risk: 3.0 - 6.0 High Risk: > 6.0	Calculated

Interpretation Notes:-
Lipid profile measures the amount of cholesterol and fats called triglycerides in the blood. These measurements give the doctor a quick snapshot of what's going on in blood. Cholesterol and triglycerides in the blood can clog arteries, making you more likely to develop heart disease.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

LIVER FUNCTION TEST (LFT) , SERUM

Test Name	Result	Unit	Bio. Ref. Range	Method
Total Bilirubin	**1.56**	mg/dL	< 1.2	Colorimetric Diazo
Direct Bilirubin	**0.40**	mg/dL	< 0.30 Neonates: < 0.6	Diazo
Indirect Bilirubin	**1.16**	mg/dL	0.3 - 1.0	Calculated
SGOT / AST	15.8	U/L	< 40	IFCC
SGPT / ALT	11.4	U/L	0 - 41	IFCC without pyridoxal phosphate activation
SGOT (AST) : SGPT (ALT) Ratio	1.39	Ratio	Refer Interpretation Notes	Calculated
Alkaline Phosphatase	60.8	U/L	40 - 129	IFCC, PNPP AMP Buffer
Total Protein	6.89	g/dL	6.4 - 8.3 Newborns: 4.6 - 7.0 Premature:3.6 - 6.0 Umbilical cord: 4.8 - 8.0	Biuret
Albumin	4.49	g/dL	3.5 - 5.2	Bromocresol green
Globulin	**2.4**	g/dL	2.5 - 4.5	Calculated
A/G Ratio	1.87	Ratio	1.0 - 2.1	Calculated
Gamma Glutamyl Transferase (GGT)	15.00	U/L	Men: 8 - 61 Women : 5 - 36	Enzymatic Colorimetric

Interpretation Notes:-
The most common liver tests include:
Liver enzymes test: Your liver enzymes include alkaline phosphatase (ALP), alanine transaminase (ALT), aspartate aminotransferase (AST) and gamma-glutamyl transferase (GGT). These are elevated when there's liver injury.
Total protein test: A total protein test measures levels of protein in your blood. Your liver makes protein, and low protein levels may indicate that your liver isn't functioning optimally.
Bilirubin test: Bilirubin is a waste product that liver deposits in bile. Possible diagnoses may include: Fatty liver disease, Toxic hepatitis, Autoimmune hepatitis, Viral hepatitis (A, B or C),Hemochromatosis, Wilson's disease, Alpha-1 antitrypsin deficiency, Primary biliary cholangitis (PBC), Cirrhosis. Liver cancer.
SGOT (AST) : SGPT (ALT) Ratio: A normal AST:ALT ratio should be <1. In patients with alcoholic liver disease, the AST:ALT ratio is >1 in 92% of patients, and >2 in 70%. AST:ALT scores >2 are, therefore, strongly suggestive of alcoholic liver disease and scores <1 more suggestive of NAFLD/NASH.

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Test Name	Result	Unit	Bio. Ref. Range	Method
THYROID PROFILE (TFT) -I , SERUM				
Triiodothyronine Total (TT3)	108.90	ng/dL	81 - 178	CLIA
Thyroxine (TT4)	7.40	µg/dL	4.5 - 10.9	CLIA
Thyroid Stimulating Hormone (TSH)	1.66	µIU/mL	0.4 - 5.5	CLIA

Interpretation Notes:-

TSH	Total T4	Total T3	Disease
Normal	Normal	Normal	None
Low	High	High	Hyperthyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
Low	Normal	High	T3 Toxicosis
Low	High	Normal	Thyroiditis, T4 ingestion, hyperthyroidism in the elderly or with comorbid illness
Low	Low	Low	Euthyroid sick syndrome; central hypothyroidism
High	Normal	Normal	Subclinical hypothyroidism; recovery from euthyroid sick syndrome

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4 AM and at a minimum between 6 - 10 PM. The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
- Unbound fraction (Free,T4/Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration.
- Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method
ELECTROLYTE SERUM , SERUM				
Sodium	141	mmol/L	135 - 150	ISE
Potassium	3.5	mmol/L	3.5 - 5.0	ISE
Chloride	100.3	mmol/L	94 - 110	ISE

Interpretation Notes:-
Too much or too little sodium can cause cells to malfunction. Lethargy, confusion, weakness, swelling, seizures, and coma are some symptoms that can occur with hyper - or hyponatremia.
Hypokalemia is most often seen when the body loses too much potassium from causes like vomiting, diarrhea, sweating, and medications like diuretics or laxatives. It is often seen in diabetic ketoacidosis, where potassium is excessively lost in the urine.
Hyperkalemia is a potentially life-threatening situation because it causes abnormal electrical conduction in the heart and potentially life-threatening heart rhythm problems. High potassium levels are most often associated with kidney failure, in which potassium levels build up and cannot be excreted in the urine.
Hypochloremia is an electrolyte imbalance and is indicated by a low level of chloride in the blood. Dehydration, fluid loss, or high levels of blood sodium may be noted and also experiencing other forms of fluid loss, such as diarrhea, or vomiting.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

CUE - COMPLETE URINE ANALYSIS (AUTOMATED REAGENT STRIP METHOD AND MICROSCOPY) , URINE

Test Name	Result	Unit	Bio. Ref. Range	Method
Physical Examination				
Volume	NA	mL		
Colour	Pale Yellow		Pale Yellow	Visual Check
Appearance	Clear		Clear	Visual Check
Chemical Examination				
pH	6		5.0 - 8.5	Dipstix-Double indicator
Specific Gravity	1.020		1.005 - 1.030	Dipstix-Ion exchange
Albumin Urine/ Protein Urine	Negative		Negative	Dipstix-Bromophenol blue
Glucose	Negative		Negative	Dipstix-Oxidase peroxidase
Ketone Bodies	Negative		Negative	Dipstix-Sodium nitroprusside
Bilirubin	Negative		Negative	Dipstix-Coupling/Fouchet
Urobilinogen	Normal		Normal	Dipstix-Ehrlich's reaction
Blood	Negative	-	Absent	Visual Check
Nitrite	Negative		Negative	Dipstix-Greiss
Leukocyte esterase	Negative		Negative	Dipstix-Esterase
Microscopic Examination				
Pus Cells (Leucocytes)	2-3	/hpf	0 - 5	Microscopy
Epithelial Cells	1-2	/hpf	0 - 5	Microscopy
RBCs	Nil	/hpf	Nil	Microscopy
Casts	Nil		Nil	Microscopy
Crystals	Nil		Nil	Microscopy
Bacteria	None seen		None seen	Microscopy
Budding Yeast Cells	None seen		None seen	Microscopy
Others	Nil		Nil	Microscopy

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Test Name	Result	Unit	Bio. Ref. Range	Method

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method
COMPLETE BLOOD COUNT (CBC) - 25 TESTS				
Haemoglobin	15.0	g/dL	13 - 17	Spectrophotometry - (non cyanide)
Total WBC Count	5120	Cells/cumm	4000 - 11000	Electrical Impedance
RBC Count	5.21	Millions/cumm	4.5 - 5.9	Electrical Impedance
Platelet Count	312	10^3/µL	150 - 450	Electrical Impedance
Packed Cell Volume (PCV)	44.7	%	40 - 50	Calculated
Mean Corpuscular Hb. (MCH)	**28.9**	pg	30 - 36	Calculated
Mean Corpuscular Volume (MCV)	85.8	fL	80 - 100	Calculated
MCHC	33.6	g/dL	32 - 36	Calculated
MPV	8.3	fL	7 - 11	Calculated
Platelet Crit	0.260	%	0.15 - 0.62	Calculated
RDW CV	13.2	%	11.5 - 14.5	Calculated
RDW-SD	40.10	fL	39.5 - 46.0	Calculated
PDW	14.9	fL	11 - 22	Calculated
P-LCR	**15.9**	%	18 - 50	Calculated
Differential Count by Flowcytometry/Microscopy				
Neutrophils	52.4	%	45 - 75	Impedance/microscopy
Lymphocytes	38.9	%	20 - 40	Impedance/microscopy
Eosinophils	2.6	%	1 - 6	Impedance/microscopy
Monocytes	5.9	%	2 - 10	Impedance/microscopy
Basophils	0.2	%	0 - 2	Impedance/microscopy
Absolute Neutrophil Count	2682.88	cells/cumm	2000 - 7000.	Calculated
Absolute Basophils Count	10.24	Cells/cumm	0 - 100	Calculated
Absolute Lymphocyte Count	1991.68	Cells/cumm	1000 - 3000	Calculated
Absolute Eosinophil Count	133.12	Cells/cumm	50 - 500	Calculated
Absolute Monocyte Count	302.08	Cells/cumm	200 - 1000	Calculated

Interpretation Notes:-
A complete blood count gives information regarding the cell types in person's blood and concentration of haemoglobin. Cells that circulate in the blood are generally divided into three types: RBC, WBC and platelets. Abnormally high or low counts may occur in physiological conditions and

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method

in diseased states and requires clinical correlation. Differential counts and RBC indices help in further understanding of the likely aetiology.
NOTE: This report has been generated by a fully automated analyser after counting thousands of cells and hence differential count may appear as decimalized numbers.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method
CALCIUM , SERUM				
Calcium	9.1	mg/dL	8.6 - 10.0	BAPTA

Interpretation Notes:-
Calcium is one of the most important minerals in your body. You need calcium for healthy bones and teeth. Calcium is also essential for proper functioning of your nerves, muscles, and heart.

Symptoms of high calcium levels include: Nausea and vomiting, More frequent urination, Increased thirst, Constipation, Abdominal pain and Loss of appetite. Symptoms of low calcium levels include: Tingling in the lips, tongue, fingers, and feet, Muscle cramps, Muscle spasms and Irregular heartbeat. Some people have pre-existing condition that may affect the calcium levels. These include: Kidney disease, Thyroid disease, Malnutrition and Certain types of cancer.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method
PHOSPHOROUS , SERUM				
Phosphorus	3.9	mg/dL	2.6 - 4.5	Molybdate

Interpretation Notes:-
Excess phosphorus will likely build up in your bloodstream if you have impaired kidney function.
Besides reduced kidney function, high phosphorus levels may be due to:

- certain medications, such as laxatives that contain phosphates
- dietary problems, such as consuming too much phosphate or vitamin D
- diabetic ketoacidosis, which occurs when your body runs out of insulin and begins to burn fatty acids instead
- hypocalcemia, or low serum calcium levels
- hypoparathyroidism, or impaired parathyroid gland function, which leads to low levels of parathyroid hormone
- liver disease

Low phosphorus levels may be due to a range of nutritional problems and medical conditions, including:

- chronic use of antacids
- lack of vitamin D
- not getting enough phosphorus in your diet
- malnutrition
- alcoholism
- hypercalcemia, or high serum calcium levels
- hyperparathyroidism, or overactive parathyroid glands, which leads to high levels of parathyroid hormone
- severe burns

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Test Name	Result	Unit	Bio. Ref. Range	Method
ALKALINE PHOSPHATASE (ALP) , SERUM				
Alkaline Phosphatase	60.8	U/L	40 - 129	IFCC, PNPP AMP Buffer

Interpretation Notes:-

High levels

Higher-than-normal levels of ALP in your blood may indicate a health concern with your liver or gallbladder. This could include a blockage in your bile ducts, gallstones, cirrhosis, liver cancer, and some forms of hepatitis.

High levels may also indicate a concern related to the bones such as rickets, Paget's disease, bone cancer, or an overactive parathyroid gland.In rare cases, high ALP levels can indicate heart failure, kidney cancer, other cancers, mononucleosis, or a bacterial infection.

Low levels

Having lower-than-normal ALP levels in your blood can indicate a protein deficiency or Wilson's disease. It may also signal malnutrition, which could be caused by celiac disease or an insufficient amount of certain vitamins and minerals.

Low ALP can also indicate a rare condition called hypophosphatasia. This causes fragile bones that can easily fracture and are slow to heal.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method
GLUCOSE - FASTING (FBS) , NAF PLASMA				
Fasting Glucose	82.4	mg/dL	70 - 110	Hexokinase

Ref.for Biological Reference Intervals: American Diabetic Association.

Interpretation Notes:-
A blood glucose test measures the glucose levels in your blood. Glucose is a type of sugar. It is your body's main source of energy. Symptoms of high blood glucose levels include increased thirst, more frequent urination, Blurred vision, Fatigue, Wounds that are slow to heal. Symptoms of low blood glucose levels include Anxiety, Sweating, Trembling, Hunger and Confusion. Blood glucose test is required to check certain risk factors for diabetes. These include Being overweight, Lack of exercise, Family member with diabetes, High blood pressure, Heart disease.

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MEDIBUDDY FULL BODY HEALTH CHECKUP

Test Name	Result	Unit	Bio. Ref. Range	Method
KIDNEY FUNCTION TEST (BASIC) - 6 TESTS				
Urea	19.2	mg/dL	16.6 - 48.5	Urease
BUN	9.0	mg/dL	7 - 18	Calculated
Creatinine	**0.60**	mg/dL	Males: 0.70 - 1.20	Modified Jaffe Kinetic
Urea/Creatinine Ratio	32.00			
BUN/Cr Ratio	14.95			Calculated
Uric Acid	4.45	mg/dL	Male: 3.4 - 7.0 Female: 2.4 - 5.7	Uricase

Interpretation Notes:-
Kidney/Renal function tests (KFT/RFT) are usually ordered when a patient has risk factors for kidney dysfunction such as hypertension, diabetes, cardiovascular disease, obesity, elevated cholesterol or a family history of kidney disease. It may also be ordered when someone has signs and symptoms of kidney disease, though in early stage often no noticeable symptoms are observed. Kidney panel is useful for general health screening; screening patients at risk of developing kidney disease; management of patients with known kidney disease.

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